June 16, 2025

Mehmet Oz, M.D.
Administrator, Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244

Dear Administrator Oz,

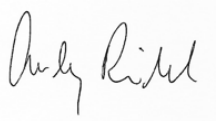
As a leading developer of certified health IT for thousands of ambulatory healthcare providers and organizations, NextGen Healthcare appreciates the opportunity to submit to HHS our public comments on the *Request for Information on the Health Technology Ecosystem.*

Over the course of more than four decades in the ambulatory health IT space, NextGen Healthcare has offered innovative solutions to our clients. As the healthcare industry shifts ever closer to value-based care, we remain steadfast in our commitment to offering interoperable, certified health IT solutions that deliver data to where it is most needed to ensure quality, efficiency, and better outcomes for all.

With this letter, NextGen Healthcare endorses the public comments submitted by our industry association, the Electronic Health Records Association (EHRA), reflecting the feedback of its nearly 30 member companies. In the attached comments below, NextGen Healthcare directly addresses the issues that have the most impact on our company and clients.

We appreciate this opportunity to share our insights on the current state and future direction of the health IT ecosystem.

Sincerely,



Andy Riedel
Senior Director, Regulatory Affairs

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NextGen Healthcare – RFI Response

Patients & Caregivers

PC-5b. Consistent and timely access to accurate patient health information requires advances in *patient matching*. Additionally, standards for health data exchange should extend to ancillary providers (e.g., labs, imaging centers), payers, and community-based organizations to ensure data completeness and interoperability across the ecosystem.

PC-8. Certified EHR technology (CEHRT) makes a wide range of clinical information readily available to patients, including diagnoses, medications, and test results. To offer patients and caregivers access to their own information, providers need to implement and use the functionality found in CEHRT. CMS should build public awareness concerning the availability and capabilities of patient-facing digital technologies and incentivize provider organizations to implement and train staff on their use, including FHIR API endpoints that allow patients to access their own health information using an app of their choice.

PC-8a. Certain social determinants such as food security, transportation access, and family support strongly influence patient health but are not typically captured in structured, exchangeable formats, nor are they fully addressed by USCDI.

PC-14a. A lack of patient awareness concerning digital health and identity credentialing, along with limited health and technology literacy, make it difficult to navigate the health IT ecosystem. Additionally, inconsistent ID verification practices across healthcare organizations create confusion and hinder adoption.

Providers

PR-3.

Technical barriers to full data accessibility. Much data in EHRs is unstructured or lacks standard mappings, limiting its usefulness in exchange and potentially leading to burden on the end of the person who is receiving the exchanged document or data. This challenge exists in all care settings, including those in urban and rural areas. Patient matching and identity resolution also remain persistent obstacles, even within standardized exchange frameworks.

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Priority level compared to other interoperability needs. In rural settings where patients often receive care from multiple, unconnected facilities and have limited access to specialists without considerable travel, efficient health data exchange is especially critical.

PR-4. The significant obstacles to providing access to administrative workflows are not necessarily technical limitations. The standards are in place to support these types of administrative processes and workflows. For example, FHIR R4 includes standard resources for Appointment and Financial Transaction that are reasonably mature and widely adopted. The challenge in enabling patient-selected, third-party products is the "trust" factor. For this to work, EHRs must allow independent applications with which they have no direct business relationship to write data into their systems. While this doesn't present an insurmountable problem, the use cases are different from allowing patient access to view or retrieve their health information via read-only transactions.

PR-6.

PR-6c. Options outside of TEFCA. Alternatives include local HIEs, patient portals, DirectTrust networks, and consumer-facing apps. However, these solutions usually require point-to-point connections, making them less scalable and comprehensive ~~than what~~than TEFCA envisions. Carequality provides a widely adopted national exchange framework, however, TEFCA is expected to enable the next level of nationwide interoperability and become the successor to Carequality.

PR-8.

PR-8a. Benefits and downsides of Bulk FHIR data. Success remains limited with deploying Bulk FHIR exports to decrease the burden of reporting on providers. EHRs are certified to support Bulk FHIR, but the lack of industry-wide use remains low. CMS should focus on creating real-world use cases and policy drivers that compel adoption before expecting it to be a primary mechanism for quality reporting.

PR-8b. Consolidation of provider quality reporting responsibilities CMS can reduce provider burden by aligning quality measures across its programs and focusing on a narrower set of core measures.

PR-12. NextGen Healthcare urges HHS to maintain existing information blocking exceptions, particularly the Infeasibility and Manner exceptions, as these exceptions allow actors to balance broad access to EHI with secure implementation. These exceptions also allow actors to exchange data efficiently without resorting to expensive, proprietary methods. Narrowing these exceptions could create barriers to interoperability and hinder innovation for smaller developers and providers. Instead of removing or modifying exceptions, we recommend continued ASTP/ONC guidance to ensure stakeholders understand their application to advancing EHI access and maintaining trust and feasibility across the healthcare delivery ecosystem.

We would support updating the Infeasibility Exception's timeline condition. A rigid ten-business-day requirement does not allow sufficient time to account for complex workflows and resolution of legitimate infeasibilities.

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We also urge against finalizing the "requestor preference" condition to the Manner Exception as it could override reasonable technical decisions made by actors and impede the progress of standards-based interoperability by forcing actors to fulfill arbitrary and unsupported preferences.

Payers

PA-5. Payers can simplify quality reporting for providers by adopting a consistent set of standards for quality measure specifications, value sets, data requirements, and calculation logic. Today, variability across payers, including within CMS programs, adds unnecessary complexity for both providers and health IT developers. Equally important is the adoption of standardized methods for provider/patient attribution to support alignment between CMS with private payers to improve accuracy of quality reporting and reduce administrative burden.

PA-5b. Aligning quality measure specifications, implementation guides, value sets, and calculation methods with CMS and other payers would enable providers and health IT developers to leverage the same infrastructure and workflows across all programs.

Technology Vendors, Data Providers, and Networks

TD-1. CMS can stimulate developer interest in the short term by promoting adoption of standardized APIs to enable seamless integration with Medicare data. Streamlined and transparent regulatory requirements would also lower barriers to entry for new digital health products targeting beneficiaries and caregivers. Longer term, CMS should implement reimbursement models that incorporate digital health products into care delivery and quality programs. This includes end-to-end alignment, establishing certification requirements and exchange standards for all involved parties (not just EHR developers and providers), along with downstream incentives for consistent use by providers, payers, public health agencies, and others.

TD-2a. CMS can further increase developer engagement by making provider directory data available via API and enabling real-time access to cost data. This type of information needs to be accessible on a real-time basis for it to be useful and actionable in the present.

TD-2d. CMS should prioritize the development of APIs that enable timely, actionable access to both cost and quality data. Providers currently lack real-time visibility into CMS-calculated cost scores.

TD-4. To encourage the use of open, standards-based APIs, CMS should ensure that standards apply on all sides of exchange, including payers, public health agencies, and other stakeholders, not just healthcare providers and EHR developers. Requiring standards only in one direction creates asymmetry, limits interoperability, and discourages investment in scalable solutions. Additionally, CMS and ASTP/ONC should avoid incorporating standards into programs before they are sufficiently mature and ensure they clearly tie to well-defined, real-world use cases. Developers are willing to adopt standards

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to meet market demands, but premature adoption without validation and operational readiness undermines confidence and increases burden.

TD-6. TEFCA offers a centralized alternative to costly and siloed HIE participation, which may improve cost efficiency for providers. The framework's unique value lies in its potential to standardize and scale national health information exchange by establishing a clear governance framework, defining use cases, and maintaining consistent technical requirements across these use cases. TEFCA's focus on existing standards, including IHE Document Exchange APIs and HL7 FHIR-based APIs, is a significant advantage, as it employs record locator services that enable identification of all a patient's data holders, thereby helping organizations and consumers communicate using uniform data structures.

TD-7. USCDI adoption has been a significant step forward for interoperability and standardization across health IT systems. By defining broad, consensus-driven data classes and elements, USCDI establishes a common foundation for the exchange of clinically meaningful data via recognized standards such as HL7 FHIR and HL7 CDA (C-CDA). This has improved the consistency and usability of health information at the point of care and across care transitions.

However, several limitations remain:

- Limited scope: USCDI does not yet cover all EHI, though this can be addressed by incrementally expanding its content.
- Inconsistency with HL7 standards: These create implementation challenges. Aligning USCDI more directly with relevant HL7 Implementation Guides would improve technical consistency.
- Update frequency: The cadence of USCDI updates can be difficult for developers and implementers to track and support, particularly when incorporated into certification and program requirements without adequate lead time.
- Lack of modularity: The "all-or-nothing" approach to adopting each version of USCDI presents challenges for specialty developers. This issue could be addressed by introducing stratified or modular implementation requirements within certification criteria. This would facilitate the certification of health IT other than EHR for the specific data they manage, which does not necessarily encompass all of USCDI.

TD-7a. USCDI does not yet encompass the full range of data elements required to support all interoperability use cases. However, ONDEC offers a valuable pathway for gradually broadening its scope to provide a more complete and standardized basis for the exchange of EHI.

TD-7b. The limitations of USCDI relate more to scope than format, although ASTP/ONC should require the use of HL7 FHIR to express the data classes and data elements. This would reduce ambiguity in translation to supporting standards (e.g., HL7 FHIR, HL7 C-CDA and others). If the goal is to standardize all EHI under a common framework in the long term, that intent should be clearly stated and supported with a deliberate, phased strategy.

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TD-7c. Continued expansion is essential to standardize healthcare data in a way that promotes consistent exchange and meaningful use. Adding new data elements inevitably introduces scoping and implementation challenges, but these can be mitigated by incorporating a modular approach within the Certification Program, allowing developers and providers to support only those data elements that align with their workflows or specialties. The timing of USCDI versions also creates challenges. Versions are published annually in July, but the corresponding HL7 implementation specifications (C-CDA and FHIR) often aren't finalized until the following May.

TD-8. Privacy and Security (P&S) criteria have been created as a consistent baseline for safeguarding health information and offers direction in the absence of HIPAA Security Rule certification. C-CDA-based criteria and Direct messaging played a critical role in the early phases of interoperability. However, as the industry shifts away from document-based exchange, the program should focus on modernizing these criteria, leveraging RESTful and HL7 FHIR. Public health criteria, especially for immunization and electronic laboratory reporting, have supported scalable reporting infrastructure and would be even more effective if not for the lack of reciprocal obligations on public health agencies (PHAs) to follow the same standards.

TD-9a. Emphasizing API-enabled capabilities, particularly HL7 FHIR-based APIs, is key to modernizing interoperability. Prioritizing these APIs over IHE Document Exchange APIs allows for more flexible and efficient data sharing, enabling developers to tailor systems to their customers' needs. However, functionality-based privacy and security criteria remain essential and should be maintained as certification evolves.

TD-9b. Focusing solely on HL7 FHIR-based APIs may be restrictive. HL7 v2 APIs are still effective for many public health workflows, and migrating away from proven infrastructure without clear ROI is unnecessary. Replacing multiple low-burden criteria with a single complex API requirement, especially if tied to immature standards, can increase the overall burden and costs. Certification must consider cost, maturity, and feasibility.

TD-9c. HL7 FHIR-based APIs already support unstructured data like faxed or scanned documents and free-text content through the Document Reference resource. Expanding USCDI to include these data types and setting clearer expectations for unstructured content would advance interoperability. However, effective interoperability requires meaningful and relevant data. While unstructured data can be valuable, applying standards and structure enhances its usability and value. We should focus on improving data quality and standardization rather than quantity to increase value and reduce cost and burden.

TD-9d. API-based data requests are valuable, but provider adoption requires meaningful incentives. Information Blocking regulations provide a compliance framework, but tangible rewards for high-quality API responses are lacking. CMS could enhance this by aligning incentives, such as rewarding automated case reporting via eCR. Policy strategies should be co-developed with providers to align API engagement with clinical workflows and resource constraints.

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TD-9e. EHRs with bulk data export capabilities can reduce provider burden under a digital quality measurement (dQM) model, but only if paired with bi-directional feedback mechanisms. Providers are accustomed to reviewing performance data through EHR-based dashboards, so if CMS or another entity performs centralized measure calculation, providers need real-time visibility into those results within their EHR environment. CMS should adopt a standardized API mechanism for pushing calculated results back into EHR dashboards to ensure continuity. Additionally, CMS should clarify the timing, frequency, and format of such feedback.

TD-11a. EHI Export should be revised to include standardized API requirements. In its current form, codified at 45 CFR § 170.315(b)(10), EHI Export has not met the goals of simplifying patient access or streamlining provider transitions between health IT systems. Lack of defined, commonly accepted standards for both data format and delivery are the greatest factors. Developers are required to document and publicize export specifications, but the complexity of health requires shared, well-understood standards. We recommend re-scoping the 170.315(b)(10) EHI Export criterion to focus on USCDI data. It should be clarified that EHI Export by itself is not a robust solution for health IT-to-health IT migration, which often requires complex mappings based on established workflows.

TD11b. The challenges with EHI Export are not in workflow execution. Developers should have the discretion to design workflows that align with their products. Standardizing workflow details would add unnecessary complexity without meaningful value. Certification should focus on ensuring the capability exists and functions reliably, not on dictating internal steps or user interfaces.

TD-11c. Should CMS consider policy changes to support this capability's use?

EHI Export capabilities are triggered by specific, infrequent use cases, such as a patient requesting a computable copy of their data or a provider transitioning to a new EHR. Therefore, CMS should not mandate their use. If existing standardized exchange mechanisms (such as FHIR or C-CDA) fulfill most real-world needs, there is no benefit in mandating the use of EHI Export for its own sake. Instead, CMS should support broader adoption of standards-based exchanges (e.g., HL7 FHIR and CDA) that already satisfy most interoperability needs. For exceptional cases requiring more comprehensive datasets, custom approaches remain viable until USCDI or FHIR profiles are mature enough to fully support “all EHI” use cases.

TD13. APIs that provide access to a patient's complete EHI can enhance patient engagement, improve care coordination, and benefit researchers and public health agencies by offering richer datasets for analytics and disease surveillance. These capabilities, combined with strong privacy protections and standardized data structures, can transform health data into actionable intelligence. CMS should offer more incentives to encourage providers to implement and maximize the functionality of these APIs, addressing their current uncertainty through payment models or conditions of participation.

Balancing value and complexity is the primary trade-off between USCDI and full EHI. USCDI offers standardized, interoperable health data, while full EHI includes vast, unstructured data that varies across

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systems and is harder to exchange and interpret. Processing full EHI requires sophisticated tools and resources, adding cost and workflow burdens without clear value for most stakeholders. Therefore, focusing on developing and adopting standards, while gradually expanding USCDI and providing flexible access to additional EHI when needed, is the most pragmatic approach.

TD-17. EHR developers can build and certify the technology, but adoption and meaningful use relies on perceived value. Providers tend not to implement new capabilities if the primary beneficiaries are other stakeholders, while providers bear the burden without direct benefit. To ensure viable healthcare data-sharing networks, CMS and ASTP/ONC must create a cohesive strategy aligning ONC interoperability goals with CMS reimbursement models, including incentives like payment reform, participation incentives, and quality program alignment.

Value-Based Care Organizations

VB-3. Essential health IT capabilities for value-based care must support care coordination, performance measurement, patient engagement, and data-driven decision-making. Standardization and consistency across CMS programs are crucial, especially in quality measurement and reporting. Value-based organizations often use multiple EHR platforms, complicating data normalization and aggregation. Inconsistent reporting requirements create unnecessary burdens and can undermine data quality. Accelerating dQM with standardized data elements and formats (e.g., FHIR-based exchange of USCDI) is vital for accurate, near-real-time insights into quality performance.

VB-3b. Bidirectional feedback between CMS and provider systems is an overlooked opportunity for improving quality measurement. EHRs are evolving to serve as the data source rather than the calculation engine in dQM models. Nevertheless, providers still expect real-time visibility into quality measure performance. Establishing standardized APIs to display performance feedback from CMS or third-party aggregators within the EHR dashboard would be advantageous to both efficiency and quality of care.

VB-4. Successful participation in value-based care arrangements depends on access to a comprehensive set of clinical, demographic, financial, and behavioral data, including diagnoses, medications, lab results, care plans, and claims data. Including access to community-based sources as part of a comprehensive solution would be advantageous. However, even the most robust datasets are ineffective without accurate patient identity matching and clear attribution. These foundational elements are essential to ensuring proper linking of data to the appropriate individuals and that providers understand which patients they are responsible for managing.

VB-8. To optimize the use of digital health products in APMs, HHS policies should extend beyond vendor certification to address the broader ecosystem, including payers, third-party application developers, and public health agencies. CEHRT requirements have driven standardization among EHR vendors, but the lack of corresponding requirements for other stakeholders hinders widespread adoption.

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